Designing for all by designing for Niche

BMVSS- Jaipur Foot

PG year 1

Moulik Nanda

Experience Design

Ishita Sarin

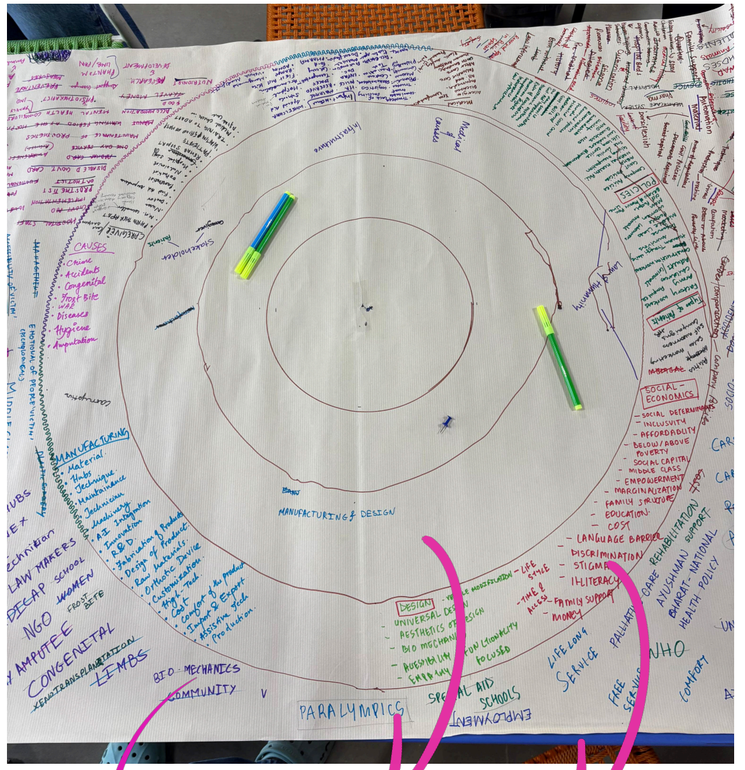
Human Centered Design

Our Journey

Initial Understanding (Before the Field Visit)

As a group of 15, we started off by sitting together and writing down everything that came to mind when we thought of Prosthetics and Orthotics on an A0 sheet of paper.

injuries, accidents, animals, medical conditions, government systems, hospitals, affordability, hygiene, comfort, long-term use, and everyday life challenges



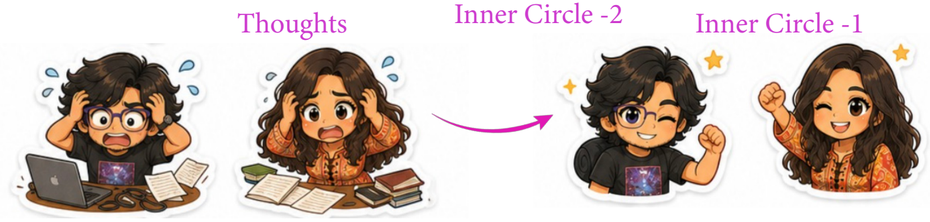
Next, we started looking for patterns in our ideas and created 9 clusters from all of our ideas with each of them falling in a cluster. then we further combined these clusters into categories.

Raw thoughts (bunch of ideas) → **Clusters (9)** (inner circle 1) → **Categories (5)** (inner circle 2)

The final 5 categories that we ended up with are as follows:

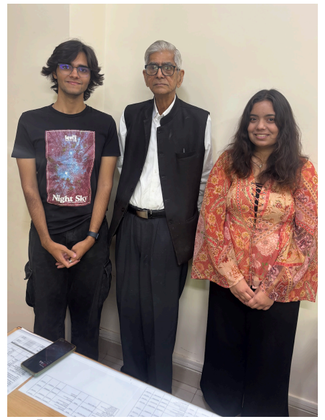
- **Manufacturing (covered by Ishita & group)**
- **Medical & Causes**
- **Infrastructure (covered by Moulik & group)**
- **Stakeholders**
- **Law & Humanity**

Link to Figjam Board (Pages) for full access of phase 1 secondary research :



From chaos to clarity

Field Visit - Bmvss (Jaipur Foot + SMS)



Day 1 (7 April)
Tuesday

BMVSS Visit + conversation with Dr. Mehta + Interviews

Day 2 (8 April)
Wednesday

All 15 people choose their different department to look into & interview people from,

Ishita - prosthetics fitting

Moulik - Robotics lab

Day 3 (9 April)
Thursday

SMS Visit + interviews + Figuring out context (Women, Repair , Dignity, visibility of services , centers exploration)

Day 4 (10 April)
Friday

Conducting interviews specific to our Context

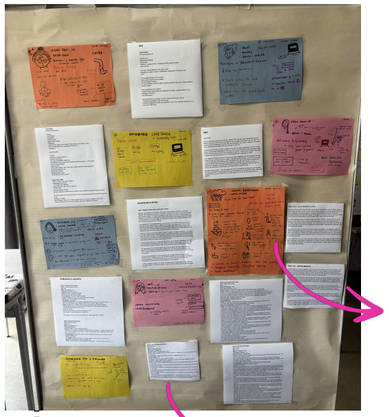
Day 5 (11 April)
Saturday

Conversation with Dr.Mehta, D R Mehta and Ms. Leela (Instalimb)

Link to Conversation with Dr. Mehta :
Link to notes

From field to feeling

Synthesising the information (After the field Visit)



Visual Interviews

Text Interviews With insights and pain points marked

Analyzing all the interviews, focusing more on the ones catering to our context of women

A demographic study of lower limb amputees in North India (2020) shows that around 23,500 new amputees are added every year, with a gender ratio of approximately 6:1 (20,200 males to 3,300 females). However, on-the-ground observations at centres like BMVSS, and confirmation with the Admin Team, reveal an even lower visible participation of women (21:1), pointing to deeper issues of access, awareness, and social barriers. This project aims to explore the gap between availability and actual access to prosthetic services, especially for underrepresented groups like women. It also looks at how dignity is experienced within these systems, not just through the prosthetic itself, but through the entire journey of seeking care. The focus is on rethinking the distribution and visibility of services so that they are more equitably accessed. This includes understanding how people discover centres, why certain centres are trusted over others, and how alternative centres can be strengthened to become more reliable, approachable, and widely known. Ultimately, the goal is to design a system where access is not dependent on chance, word-of-mouth, or geography, but is structured in a way that ensures dignity, awareness, and equal opportunity for all individuals in need of care.

Coming up with design brief

Insight Tracker

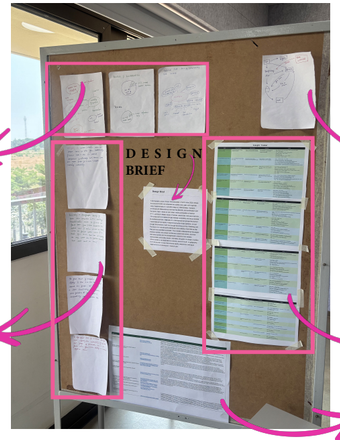
Insight	Patient experience	Interview Notes	Anecdotes	Source
1. Discovery is largely informal (word-of-mouth dominates)	Most patients learned via village networks or relatives	Word-of-mouth is primary discovery channel, even after years of disability	Strong repetition: "someone told me", "met someone", camps, relatives, YouTube occasionally	• Jibran & Rushail – Firoz (Moradabad), Sanjay (UP), Munna (Bihar) • Tanay – Firoz, Munna • Urvi – Seema (Guntur), Hanuman (Maharashtra) • Subrat – Ramesh – Kumar, Rashid • Nidi – Manipur case • Sumandhira & Anukampa – Hanena, Sanjay • Iriya & Medha – Sanjay (Arena)
2. Lack of awareness of other centres	Patients travel long distances despite other options	People ignore closer centres (Varanasi) due to trust/familiarity gaps	Many unaware of nearby centres; confusion about system navigation	• Tanay – Indrajeet Gupta, Vinay Kumar • Sumandhira & Anukampa – Lagan Singh, Ranjeet • Nidi – Aarti (Bhopal), sister farmer (film-based awareness only) • Urvi – Hanuman (asked about Mumbai centres), Danish • Ishita – caregiver father (multiple hospital-hopping), Maharashtra man • Subrandi & Zohra – general observation (no knowledge of local repair centres)
3. High travel burden (cost + distance)	Multi-day train journeys; patients travel alone	Travel is expensive → pushes users to inferior alternatives first	₹1500 travel cost is significant; multi-day stays; multiple centre movement adds cost	• Iriya & Medha – Harmeetsunder Joshi • Tanay – Firoz Dhok, Vinay Kumar • Sumandhira & Anukampa – Munni, Pooja (3 days), Mukesh • Subrandi & Zohra – Hanena (3 days), Ashok (~3 days), Rashid (~2 hrs) • Urvi – Hanuman (26-hour journey) • Nidi – multiple long-distance train travellers • Ishita– 3 Days
4. System navigation is confusing (token, process, flow unclear)	Some procedural understanding but no clarity beforehand	First-time users anxious about process	Repeated confusion: where to get token, what next, delays, multiple visits required	• Ishita – UP woman (token confusion, no visibility of process) • Iriya & Medha – Sanjay (first-time doubts about process) • Nidi – Maharashtra male (token removed incident) • Sumandhira – Aashima (didn't know where to go)& husband, Munni, Aarti Lodhi • Tanay – Rabi (uncertainty about fitting journey)
5. Long waiting + lack of support infrastructure	Patients often alone; prefer short stays due to discomfort	Waiting periods with no support (family leaves early)	Patients staying outside, without food/ accommodation in extreme cases	• Tanay – Firoz Dhok • Sumandhira & Anukampa – Naushad Ali • Iriya & Medha – Harmeetsunder Joshi • Jibran & Rushail – Firoz (waiting alone after family left) • Subrandi & Zohra – people sleeping outside, inconsistent food • Urvi – Hanuman (no accommodation) • Ishita – multiple (fatigue, no rest when otherwise)
6. Prosthetic = complete solution (ecosystem issues remain)	Mobility improves, but lifestyle still restricted	Employment, social dignity, and independence still unresolved	Many still rely on crutches, tricycles, or jugaad mobility	• Iriya & Medha – Sanjay Singh • Sumandhira & Anukampa – Mukesh Yadav • Jibran & Rushail – Rushail (Employed) • Subrandi & Zohra – Kumar (limited job ability) • Tanay – Rakesh (uncertain return to work);Hattor Singh • Urvi – Mira (life improved but still constrained) • Ishita Moulik – Lady not able to dance, accessorize

Tracking all the insights

Annotated photos for better understanding of the pain points

Paper Name	Link	Important points from the paper
A Report on Amputees in India	https://www.oandplibrary.org/op/pdf/1986_01_018.pdf	The pie chart shows the distribution of approximately 424,000 amputees in ind 268,000 (63%) – are rural males, making them by far the largest segment. Rura 70,000 (17%), meaning rural and urban males together represent about 80% of Overall, males outnumber females roughly 4:1, and rural areas account for abo
INVISIBLE STRUGGLES: EXPLORING CHALLENGES FACED BY WOMEN WITH AMPUTATION IN INDIA	https://pmc.ncbi.nlm.nih.gov/articles/PMC11609910/pdf/tpoj-7-1-64002.pdf	1. gender plays a role in prosthetic access and fitting, as fewer women (42.9%) (68.6%). A This disparity may stem from concerns related to the appearance of 2. In our population, Road Traffic Injuries (RTIs) emerge as the leading cause of laws and an 3. Increasing population. 3. In patriarchal societies like India, there is often a tendency to prioritise men' women's genuine concerns as "emotional" or "natural phenomena".5 This soci women as compared to men. Additionally, women with disabilities face a dual physical disability. This discrimination, combined with lower literacy rates, sign 4. They often fear abandonment by their husband or in-laws or have negative daughters do not consent for amputation fearing their uncertain future. Conse more vulnerable than both able-bodied women and disabled men.
Gender differences in amputation outcome	https://sci-hub.kmp.top/10.1080/09638280701254095	1. More men than women were successfully fitted with a prosthesis (42.9% vs 2. Only gender and social isolation were significant factors contributing to the multivariate model, only gender was independently significant. This indicated but that people who live alone do less well because they tend to be women, n 8.8% of variance.
IIT Guwahati creates affordable prosthetic leg for Indians that allows deep squatting, cross-legged sitting	IIT Guwahati creates affordable prosthetic leg for Indians that allows deep squatting, cross-legged sitting - India Today	
Focusing Research Efforts on the Unique Needs of Women Prosthesis Users	Focusing Research Efforts on the Unique Needs of Women Prosthesis Users	1. Women with limb loss experience greater dissatisfaction with prosthetic fit prosthetics solutions to accommodate the desire of women to wear different f footwear is important to attire and hence community participation. Despite th research, which limits available information to guide the rehabilitation process 2. Women represent a growing portion of the population with lower-limb loss are limited prosthetics solutions to accommodate the needs of women with li optimal mobility outcomes.
Psychological Flexibility in People with Lower Limb Amputation among Males & Females in Indian Population	Psychological Flexibility in People with Lower Limb Amputation among Males & Females in Indian Population	The result of the studies shows that there was a gender difference in context females with lower limb amputation are psychologically less flexible than male psychological inflexibility is a major problem post amputation.more commonly psychological disturbances which can be a painful memory leading to a less sa to the male dominant society therefore it will be helpful to understand the m them a better & more satisfactory life.
Identifying and Acknowledging a Sex Gap in Lower-Limb Prosthetics Research: A Review	Identifying and Acknowledging a Sex Gap in Lower-Limb Prosthetics Research: A Review	
Depression and anxiety symptoms after lower limbamputation:	https://journals.sagepub.com/doi/epdf/10.117	

Secondary research to validate the information and get more insights



Stats and Maps to Understand Women and Prosthetics

Service Ideation (Stage - 1)

Context

Insight Tracker

Secondary Research Phase 2

synthesis and early ideation

[Link to Secondary Research Phase 2]

[Link to Insight Tracker]

[Link to ALL interviews]

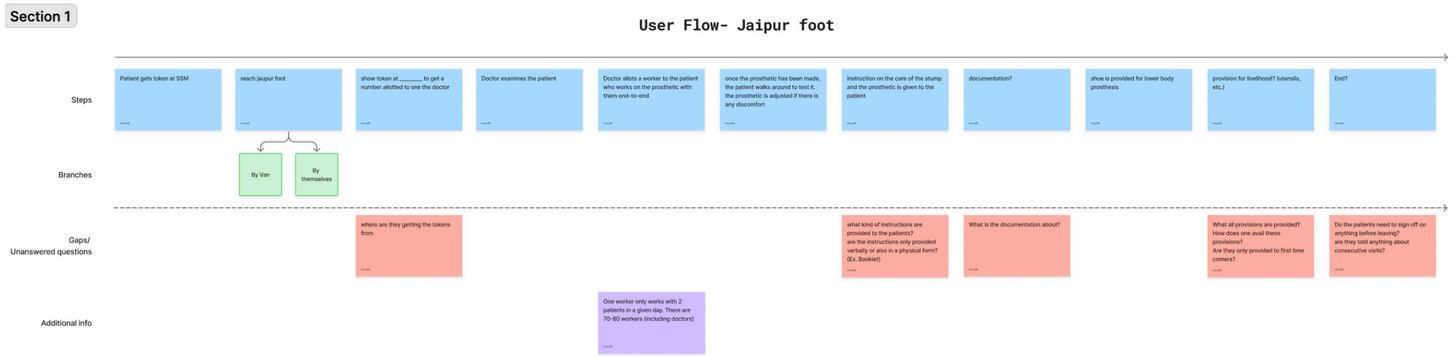
[Link to ALL Women interviews]

[Link to PDF of colored sheet interviews]

From data to direction

Customer Journey & Pain Points

To understand the user experience across the system and in daily life



Journey of a customer from start to end in Jaipur Foot

Abled women vs Women with Prosthetics

The second journey map **compared** a **rural homemaker with prosthetics to an able-bodied woman** following the same daily routine, revealing significant differences in effort, comfort, and independence. It highlighted that while prosthetics enable mobility, they often come with added physical strain, fatigue, and social limitations.

A woman living in a semi-rural area wakes up early in the morning. She freshens up, bathes, and gets ready for the day. She prepares breakfast for her family and helps her child get ready for school, seeing them off at the gate.

Bathing, Getting Ready

	Woman with Prosthetic	Able-Bodied Woman
Experience	• Needs to wear prosthetic, • manage stump care, • balance while bathing, • difficulty in using washroom, • Don't think prosthetic as a part of her	Moves naturally without preparation, easy to use washroom
Pain Points	Sweat, irritation, fear of slipping, time-consuming	Body pain maybe (other reasons)
Emotion	Slight stress, body awareness	Neutral, effortless
Opportunity	Easy-wear prosthetic, anti-slip bathing solutions, breathable materials	—

Breakfast & Sending Children to School

	Woman with Prosthetic	Able-Bodied Woman
Experience	• Limited standing time, • needs support while cooking	Continuous, fluid work
Pain Points	Fatigue, imbalance, restricted movement	Minimal fatigue
Emotion	Pressure to keep up, quiet strain	Routine responsibility
Opportunity	Load-support kitchen tools, energy-efficient prosthetic	—

Once back, she does her daily household responsibilities, cleaning, washing clothes, and preparing lunch. The work continues through the afternoon, balancing multiple tasks at once.

Household Work

	Woman with Prosthetic	Able-Bodied Woman
Experience	• Breaks tasks, alternates between sitting & standing, • foot broken & loose(needs to go to jaipur again , not sure of nearby centers)	Completes tasks continuously
Pain Points	Cannot lift heavy loads, fatigue, dependence	General tiredness only
Emotion	Guilt, compromise	Control, independence
Opportunity	Assistive tools, hybrid indoor mobility, ergonomic workflows	—

Cooking(Later in the day)

	Woman with Prosthetic	Able-Bodied Woman
Experience	• Increased discomfort, • sweating inside socket	Normal workflow
Pain Points	Skin irritation, heaviness, exhaustion	Low fatigue
Emotion	Endurance mode	Neutral
Opportunity	Ventilated sockets, lightweight materials	—

By evening, she gets ready to step out and it starts to rain. It is Tuesday, and like every week, she attends prayers at the local temple, followed by a small gathering in her neighborhood. After spending some time socializing, she returns home, finishes any remaining work, and prepares to rest.

Social Visits

	Woman with Prosthetic	Able-Bodied Woman
Experience	• Difficulty sitting on floor, clothing constraints (saree, footwear), • wet Prosthetic, • cant have mahendi and aesthetic elements, • slippers slip off without her noticing , • not able to dance , • needs to remove prosthetic for tasks like using restrooms and sitting on the floor but cannot in public	Fully participates
Pain Points	Posture limitation, social discomfort,	None
Emotion		
Opportunity		

Night Time

	Woman with Prosthetic	Able-Bodied Woman
Experience	• Removes prosthetic, • cleans, rests due to fatigue	Simply rests
Pain Points	Swelling, pain, maintenance effort	None
Emotion	Relief + exhaustion	Relaxation
Opportunity	Low-maintenance design, comfort-focused wear	—

Lived Experience of people

Link to storyboard:

Link to ALL Customer Journey Maps (in different pages):

Ideas Iterations

Phase 1
(Exploring)

After synthesising our research, we explored ideas freely without filtering, capturing everything from quick fixes to system-level interventions across multiple themes. These ideas were then categorised and mapped to reveal patterns, overlaps, and deeper opportunity areas.

Phase 2
(Categorization)

Building on the initial pool, Phase 2 focused on making decisions by filtering them based on feasibility within the given timeframe. Using a colour-coded system, we narrowed down the scope to ideas that were not only relevant but also realistic to prototype and develop further.

Phase 3
(Execution tools)

In Phase 3, we shifted focus to how the ideas could be executed, categorising them based on the tools and software required for development and representation. This helped us visualise how each idea might translate into real-world scenarios while also identifying potential challenges early on.

Phase 4
(Ideas Generated)

In the final stage, we continued exploring multiple ideas, but shifted focus from breadth to depth, selecting only two concepts to prototype based on relevance, feasibility, and potential impact. The chosen directions were the Footwear Lock System and the Home-Based Prosthetic Service Model

Link to Ideas Phase 1 : exploration

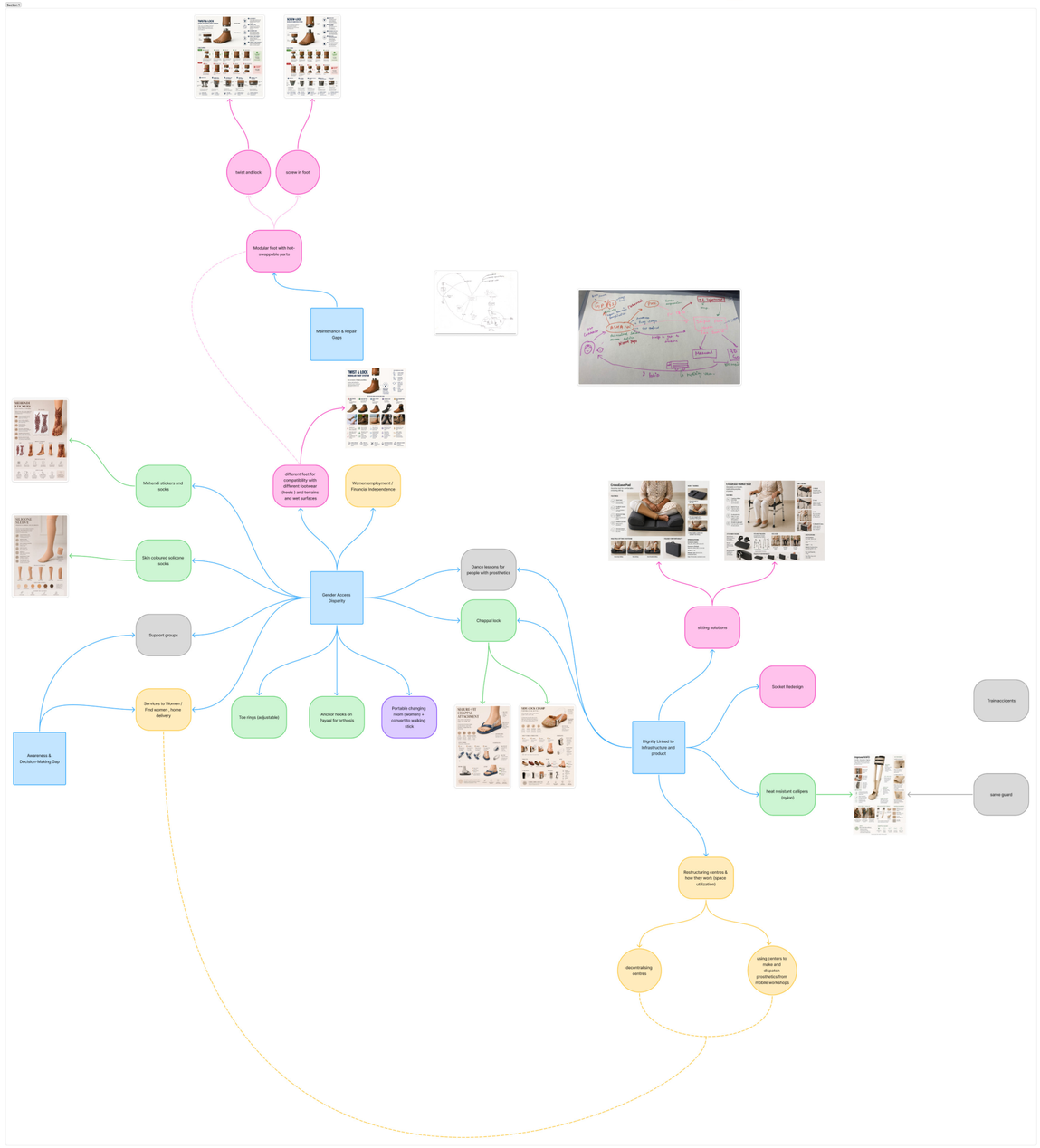
Link to Ideas Phase 2 : categorization

Link to Ideas Phase 3 : tools execution

Link to Ideas Phase 4: Generated

Running to catch the perfect idea

Ideas Final Map



[Link to Ideas Phase 1 : exploration]

[Link to Ideas Phase 2 : categorization]

[Link to Ideas Phase 3 : tools execution]

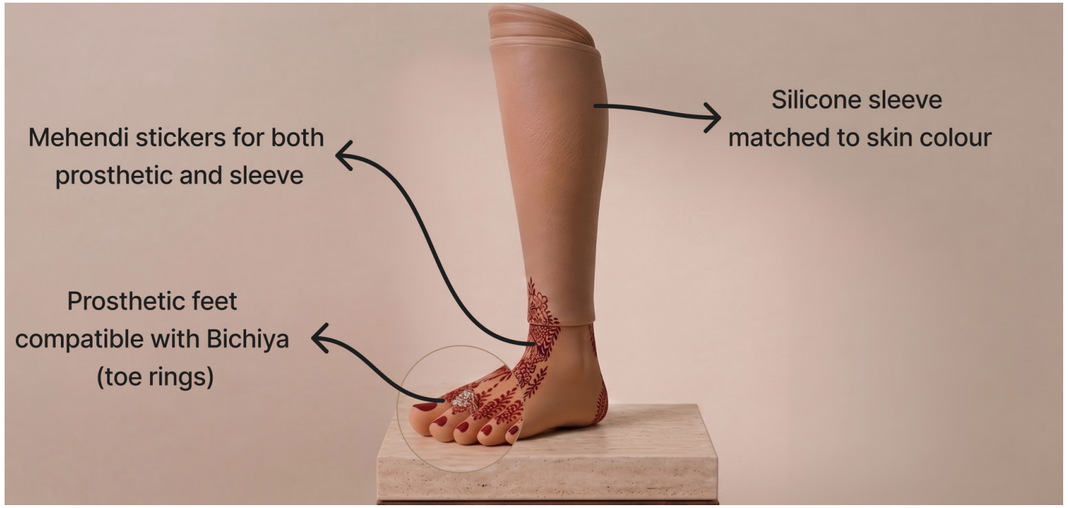
[Link to Ideas Phase 4: Generated]

Running to catch the perfect idea

Shortlisted Ideas

Mehendi stickers are introduced to enable cultural expression on prosthetics, allowing women to participate in traditions with confidence.

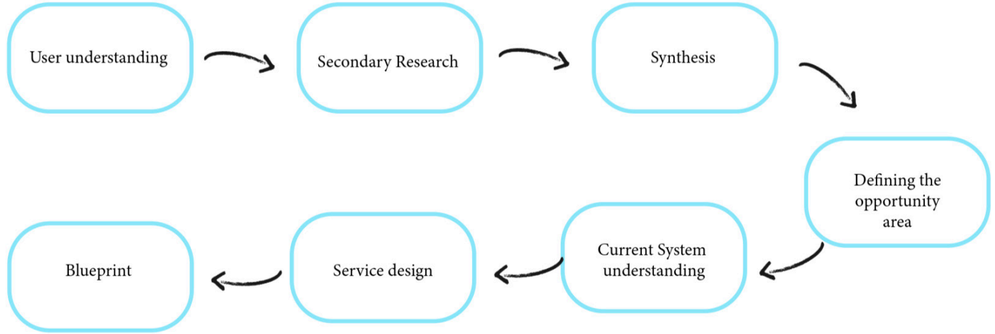
A skin-tone matched silicone sleeve is designed to create a more natural appearance, improving comfort and social confidence.



Potential concept of combining these ideas with further iterations for value addition (Bichiya compatibility)

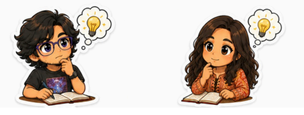
Service design

Care that reaches you, where you are

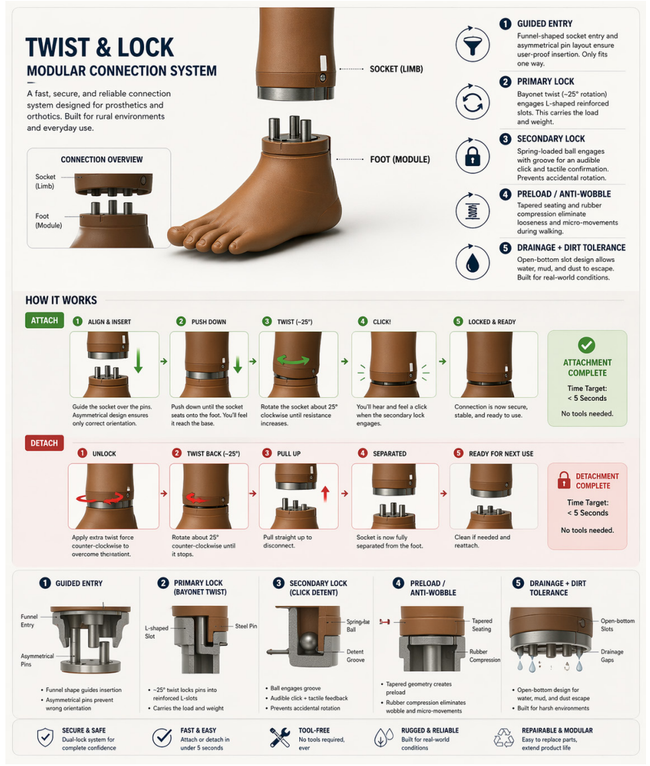


A way to provide home based services to women and children using Jaipur foot technology (3d scan , mobility van , manual , centers)

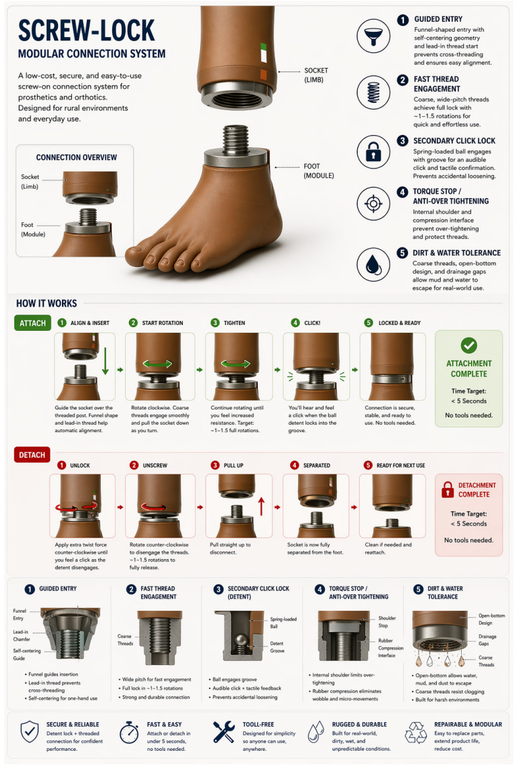
[Ideas Images]



Shortlisted Ideas



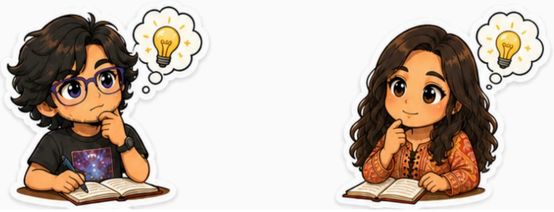
A twist &lock system is introduced to enable quick attachment and detachment, making prosthetics more modular and easy to repair.



A screw-lock mechanism is designed to provide a secure yet simple connection, providing a more cost effective alternative to the twist and lock mechanism

A set of interchangeable components taking advantage of the twist & lock mechanism, adapting to different terrains and activities.

An improved KAFO design is created with insulation and protective layers to enhance comfort and prevent fabric entanglement.



[Ideas Images]

From insights to possibilities

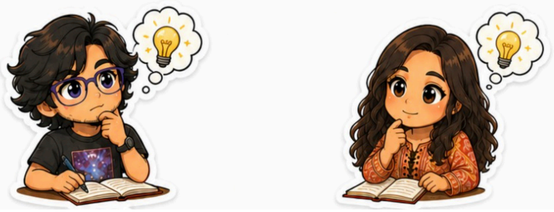
Shortlisted Ideas

A chappal lock mechanism is developed to secure footwear in place, preventing slipping and improving walking stability.

A side-lock clamp system is created to adapt prosthetics for different footwear styles, enabling greater choice and cultural compatibility.

A support pad is designed to enable comfortable cross-legged sitting, aligning prosthetic use with floor-based living practices.

A foldable walker seat is designed to provide immediate resting support, reducing fatigue during prolonged mobility.



From insights to possibilities

[Ideas Images]

Final Concepts(2 pronged approach)

How might we address the lack of access to prosthetic care for women who cannot step out due to social and mobility constraints?

Service Design

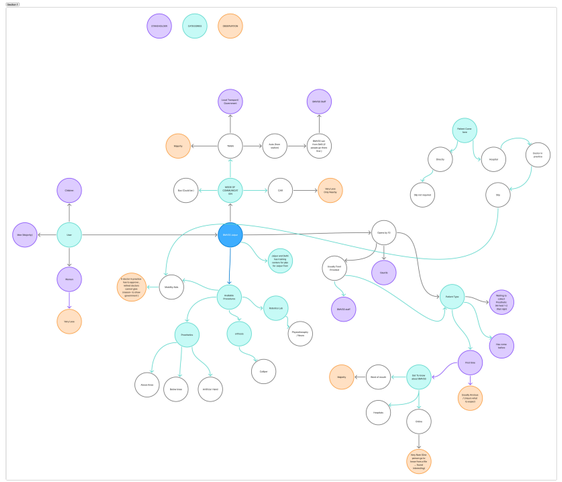
How to enhance the experience of women with prosthetics (personalization)?

Chappal Lock

From possibilities to impact

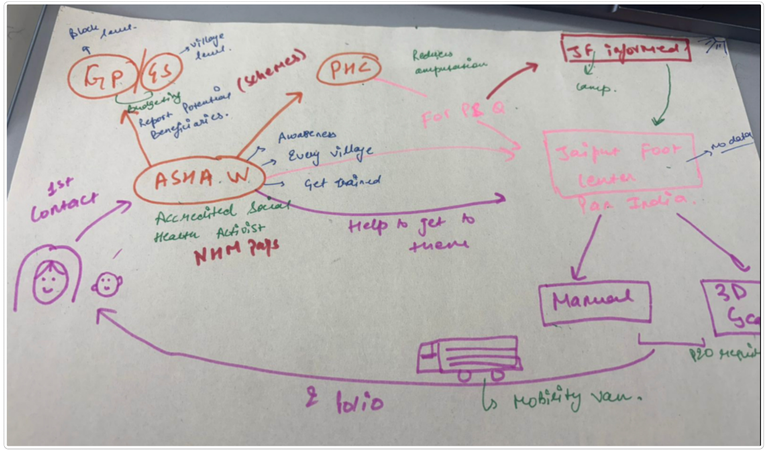
Prototype 1 - Service Design

How might we address the lack of access to prosthetic care for women who cannot step out due to social and mobility constraints?



Current System understanding of BMVSS

Current state of mobile workshops and 3d scanning for prosthetics



Proposed service design model V1

The proposed service model reimagines prosthetic care as a decentralized and continuous system, moving away from the current one-time, center-based approach. It incorporates existing community networks such as ASHA workers, Primary Health Centres (PHCs), and local governance bodies to identify potential users early and create a more structured pathway to care. Instead of relying solely on individuals to travel long distances to centralized facilities, the model also introduces assisted access through mobility support, coordinated transportation, and localised interventions such as camps(no center available) and mobile workshops(center available). The inclusion of technologies like 3D scanning further enables preliminary assessments and data collection to happen faster and closer to the user, reducing delays and repeated travel. The model also emphasises better coordination between stakeholders by introducing clearer communication flows and basic tracking mechanisms, ensuring that users are not lost in the system after initial identification. This helps streamline the journey from awareness to fitment, making the process more transparent and accessible.

Ecosystem Mapping of Jaipur foot :

Link to Service Model mapping :

Link to Blueprint :

Let's make it happen

Proposed Solution

[Ecosystem Mapping of Jaipur foot :]

[Link to Service Model mapping :]

[Link to Blueprint :]

[Link to ALL Women interviews :]

[Link to Secondary Research Excel :]

[Link to PDF Secondary Research :]

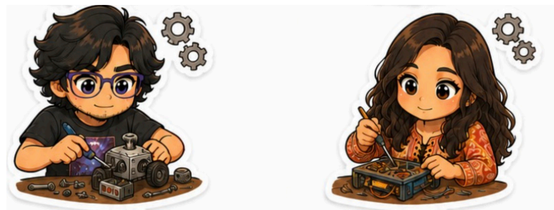
Designing Dignity Service blueprints

Service Design - make prosthetics and orthosis Available for all especially women and children

PHASE	Asha Worker goes to the households	Gives Beneficiary lists to GP/GS and PHC	Asha worker tries to take the careseeker to Jaipur foot(manual)	If care seeker cannot go , a mobile transportation is done (3...	If there is no jaipur foot center nearby, PHC/Gp requests for a camp
PHYSICAL EVIDENCE	Asha Travel- van , car, bike, ASHA register / notebook, need assessment form	Paper beneficiary list Registers / handwritten records, stamps , forms	Travel, Bags, documents, Physical effort	Van/ vehicle , Medical transport setups, scanning equipment	Temporary camp setup Tools, materials Space, Queues, crowd
CUSTOMER ACTIONS	Care Seeker • Receives ASHA at home • Shares condition (injury, amputation, difficulty walking) • Expresses needs or struggles • May or may not know about prosthetics • Asks questions hesitantly • Often influenced by family members	• Passive role (user is not actively involved) • Waits for updates • Relies on ASHA / local system • Often unaware of what is happening/ updates	• Decides whether to travel • Family support • Faces fatigue, discomfort	• Adjusts to schedule • Limited control over timing	• Waits for camp announcement • Travels locally • Gets assessed and fitted • Waits in queues
LINE OF INTERACTION					
FRONTSTAGE ACTIONS	• ASHA observes patient condition • Asks basic health related questions anf family related support • Gives verbal suggestions • Builds trust through familiarity	• ASHA submits names • GP/PHC acknowledges list • Officials may verify or ignore • No direct communication with user	• ASHA may guide or accompany • Provides verbal instructions	• P&O required for 3d scan • Coordinator to arrange to and fro of vehcile • ASHA informs users • Staff assists boarding	• Doctors + technicians interact • Measurement + fitting • Instructions given
LINE OF VISIBILITY					
BACKSTAGE ACTIONS	• ASHA notes potential beneficiary • Can help with pwd cards • Takes the list to GP/GS	• List aggregation • Basic filtering (sometimes) • Prioritization (sometimes) • No tracking system as such • Data may get lost or delayed	• Transport coordination • Arrangement , bookings	• Transport coordination • Route planning • Coordination between local bodies + Jaipur Foot • Capacity planning	• Camp planning • Resource allocation • Staff deployment • Bulk processing
LINE OF INTERNAL INTERACTION					
SUPPORT PROCESSES	• Government ASHA network • General health training	• Government administrative system • Paper-based workflows • No digital integration • No prosthetic-specific pipeline	• Informal travel planning • Family dependency	• P&O required • Funds • Ad-hoc logistics	• Government approvals • Donor funding
Possible Key Issues	• Awareness depends on ASHA knowledge • No systematized identification • Highly inconsistent outcomes	• Data flow is slow and unreliable • No feedback loop to user • No ownership of next step	• Physical burden • coordination • accessibility	• P&O required • Delivery and changes • might not be done in one day	• Episodic (not continuous) • No follow-up after camp - might need to go to nearby centers • Overcrowded, rushed

A key focus of the model is on post-fit continuity, addressing one of the most critical gaps in the existing system. It proposes utilising the mobile workshop model for home-based follow-ups, minor adjustments, and contextual training that helps users adapt the prosthetic to their daily lives, including activities such as sitting, walking on different terrains, and managing hygiene. **This is particularly important for women, who often face additional social and mobility constraints and whose needs are not adequately addressed in current systems.**

By integrating community-level support with centralized expertise, the model shifts prosthetic care from an episodic service to an ongoing support system. It aims to improve not just access, but also long-term usability, comfort, and user confidence, ensuring that prosthetic care extends beyond fitment and becomes embedded in the user's everyday life.

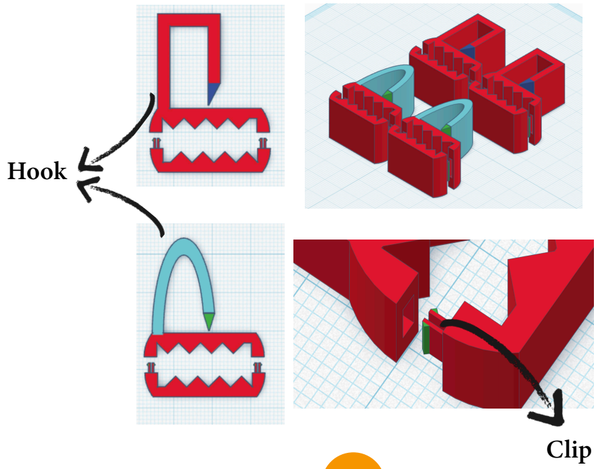


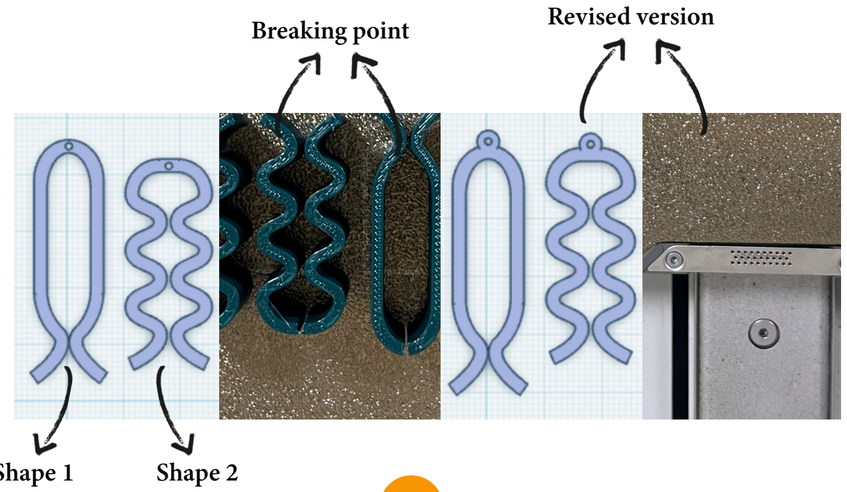
Let's make it happen

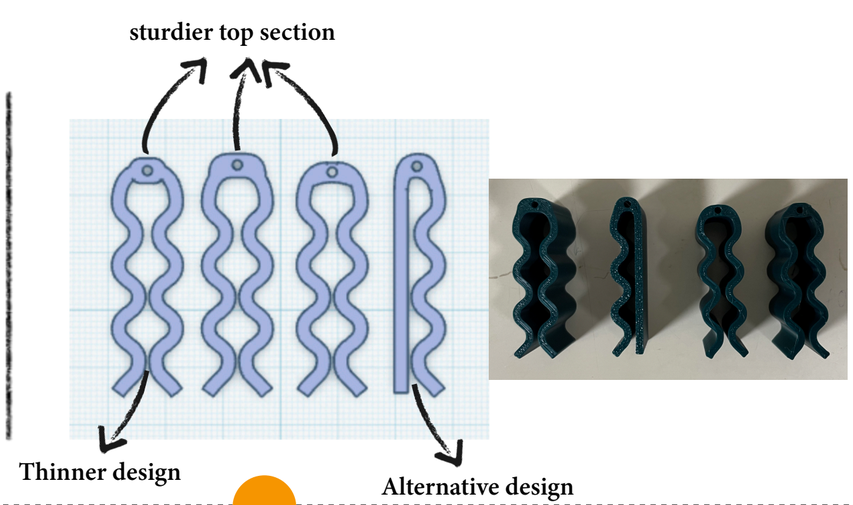
Prototype 2 - Chappal & Jutti Lock

How to enhance the experience of women with prosthetics (personalization)?

while the service tackled the problem of women not being able to get the prosthetics, the women who already had the Jaipur had problems and pain points of their own, one of them being the lack of footwear options for women. to tackle this problem we took our chappal and jutti lock concept into the prototyping phase.







Prototype 1

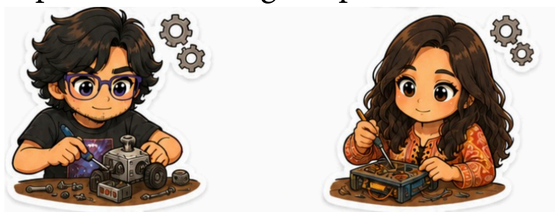
First prototype, clips onto slipper strap and has hooks to attatch the back strap to.

Prototype 2

Iterated to a version that would slide into the strap of a chappal and then be secured to it using a rubber band/O ring, then iterated further to a version with curves and grooves in it that would hold on to any footwear using friction. Also added a hole to the clip for a metal ring that would act as the hook but it made the structure too fragile in the first version and broke instantly. To fix this we made a revised version of this prototype with a cylindrical extrusion for the hole.

Prototype 2

The third and final prototype focused on small changes and quality of life improvements with a thinner shape, a thicker extrusion at at the top for structural integrity and an alternative version of the clip with one flat side for additional comfort for people without prosthetics using the product as an assistive device.



Let's make it happen

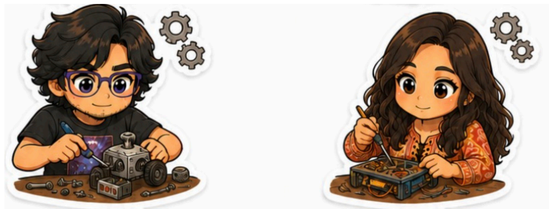
Proposed Solution

Current prototype/ proof of concept

Final proposed solution

Our current proof of concept uses paper clips as a makeshift metal ring and an adjustable elastic strap attached to it to create our chappal lock mechanism.

The product held up well in our limited user testing and seems to be ready to be taken to the final stage where we will use an elastic material like polypropylene for the clips with a grippy texture on the inside, stainless steel metal rings and a transparent adjustable strap made of TPU going through the back of the foot. This solution will be universal and work on other kinds of footwear like mules and juttis, giving women with prosthetics the freedom to choose and wear whatever footwear they like without making any permanent modifications to it.



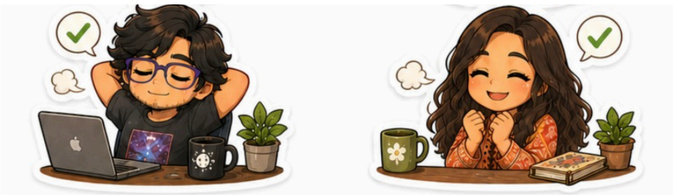
Let's make it happen

Future Scope

Chappal Lock : The proposed solutions, both at the product and system level, have the potential to expand beyond their current scope and become more inclusive and scalable. The chappal lock concept can be adapted for a wider range of users, including elderly women and even able-bodied individuals who face issues with unstable footwear, making it a more universal solution rather than being limited to prosthetic users. It can also evolve into multiple variations in terms of design, materials, and colors, allowing users to choose based on personal preference and cultural context, while maintaining the same core functionality of providing stability and grip.

Service : At a system level, the service model can be scaled across regions by integrating more deeply with existing public health infrastructure such as ASHA workers, PHCs, and local governance bodies. It can further incorporate digital tools for tracking, coordination, and communication, improving efficiency and continuity of care. The model can also expand to include training for local workers, enabling them to provide basic support, follow-ups, and guidance at the community level.

Together, these interventions can evolve into a more comprehensive ecosystem that not only improves access to prosthetic care but also enhances long-term usability, independence, and inclusion across different user groups and contexts.



Addressing the bigger picture

Links

Link to Ideas Phase 1 : exploration

Link to Ideas Phase 2 : categorization

Link to Ideas Phase 3 : tools execution

Link to Ideas Phase 4: Generated

Link to Secondary Research Excel :

Link to PDF Secondary Research :

Link to Blueprint :

Link to Secondary Research (FULL ACCESS) :

Link to PDF'S secondary Research :

Link to ALL Women interviews :

Ecosystem Mapping of Jaipur foot :

Ideas Images

Link to Figjam Board (Pages) for full access of phase 1 secondary research :

Link to notes :

Link to Conversation with Dr. Mehta :

Link to ALL interviews :

Link to ALL Women interviews :

Link to PDF of colored sheet interviews :

Link to Insight Tracker :

Link to Secondary Research Phase 2 (FULL ACCESS) :

Link to ALL Customer Journey Maps (in different pages):

Link to storyboard:

Link to Service Model mapping :

Check them out for details

THANK YOU

Designing for all by designing for Niche

Goodbye, until next time.